

Recurrent Angioedema Associated with Prindalol Rechallenge: A Case Report

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Abstract. A fictional case of recurrent angioedema following inadvertent rechallenge with Prindalol (fictional ACE-inhibitor analog) is described, with brief comment on the risk of rechallenge after a first unrecognized reaction.

Case Presentation

A 71-year-old fictional male patient presented with lip and tongue swelling six hours after taking Prindalol 10 mg, prescribed two days earlier for hypertension following a hospital discharge. He had experienced a milder, self-resolving episode of lip swelling approximately one year prior while on the same medication, which had not been recognized as drug-related at the time and the medication was continued.

On this presentation, the swelling was more pronounced and involved the tongue, prompting an emergency department visit. He was treated with antihistamines and a short course of corticosteroids, with resolution over 24 hours without airway compromise. Prindalol was permanently discontinued and the reaction was documented clearly in his record with an allergy alert.

Brief Discussion

This case underscores the risk of rechallenge with ACE-inhibitor-class agents after an unrecognized first angioedema episode, and the importance of specifically asking about prior facial or oral swelling when prescribing this drug class, even when a prior episode was mild and self-limited.

References

1. Petrenko V. Angioedema recognition in primary care prescribing. Fictional Allergy Immunol Notes. 2025;3(1):9-11.